

COMMITMENT, AND HOW EQUITY IS EARNED

A response to Medbury's review of the sharing framework, and an answer to the question of what keeps each party in

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Prepared by Consult for Africa. Draft for discussion. Private and confidential.

1. WHERE WE HAVE ALREADY LANDED

- 1.1 Medbury's review is a good one and most of it is right. This paper concedes eight of the ten questions outright, answers the other two, and then addresses the point that actually matters, which is the one put at the end of the exchange: what prevents Dr Kpaduwa or CFA from walking away.
- 1.2 Two concessions have already been made in the conversation and they change the arithmetic, so they are recorded here first. **CFA will hold 10 per cent, not 15. And CFA will charge an arm's length market rate for management rather than a discounted one.** The second of those disposes of the review's strongest objection, which was that the difference between a theoretical market fee and a negotiated fee should not automatically become permanent equity. It should not, and it no longer does. CFA is paid the market rate for management and takes no equity for management at all.
- 1.3 Run on that basis, and with the brand valued on the revenue the name can be shown to attract rather than on the whole book, the framework produces **60 / 30 / 10**. Medbury's own stated comfort zone was 55 to 60, 30 to 35, and 8 to 12. The three numbers sit inside it. **The negotiation over percentages is therefore substantially closed**, and what is left is the architecture, which is where Medbury said it wanted to start.

2. THE QUESTION, AND WHY IT ANSWERS ITSELF

- 2.1 The concern is that sweat equity leaves the door open, because a party that has put no tangible asset in feels little pain in walking out. That is correct as the first framework was drafted, and the fault is ours rather than anybody's character.
- 2.2 **The first framework valued three years of contribution and then awarded the equity for all of it on day one.** Nobody would design that deliberately. It pays in advance for work not yet done, and a party paid in advance is, by construction, free to leave. The problem is not that the contributions are intangible. It is that the timing of the award and the timing of the delivery were never aligned.
- 2.3 Align them and the problem dissolves without a single restrictive covenant. That is the whole of this paper. It is also, usefully, not a lock imposed on anyone. It is the ordinary consequence of the valuation method all three parties have already accepted.

3. TWO CLASSES OF EQUITY

- 3.1 Every contribution is classified by when it is delivered, and the equity for it is granted at that point and not before.
 - (a) **Founding equity** is granted for value that is delivered and verifiable at or near completion: money actually funded, a fit-out actually installed, protocols actually written and handed over, a company actually incorporated and licensed, a team actually hired, systems actually working. It is granted on delivery, not on promise, and once granted it is unconditional.
 - (b) **Earned equity** is granted for value delivered over time: clinical work, training, the use of a name, a referral flow, an operating service. It vests as the contribution is actually made, measured against what actually happens rather than what was forecast.
- 3.2 The classification is not a judgement about who is trusted. It is a statement about when a thing arrives.

4. WHAT THAT PRODUCES, AND THE ANSWER IT GIVES

- 4.1 Applying the classification to the contributions already agreed, and with everyone now paid a market rate in cash for services rendered:

Party	Founding	Earned	Total	Equity	Of which vests
Medbury Healthcare Group	126	109	236	54%	46%
Dr Chinwe Kpaduwa	45	108	153	35%	71%
Consult for Africa	18	26	44	10%	60%
Total	189	244	433	100%	

- 4.2 Figures are NGN millions. The last column is the answer to the question that was asked, and it is worth reading slowly.

- (a) **71 per cent of Dr Kpaduwa's equity is earned rather than founding.** It vests only as she licenses the name, trains the team and generates demand. If she steps away after eighteen months, she keeps what she has delivered and the rest never vests. Nothing restrains her, and nothing needs to.
- (b) **All of CFA's equity is founding.** Because CFA now charges a market fee for management, none of its 10 per cent is for future service. It is for incorporation, regulatory establishment, recruitment and systems, each of which is a deliverable with a date. It vests on delivery of those milestones, and if CFA walks before delivering them it receives nothing for them.
- (c) **54 per cent of Medbury's equity is founding,** which is right, because its money is spent and at risk from the first day and cannot be handed back.

- 4.3 So the party whose commitment was most in doubt holds the equity that is most contingent, and the party bearing the cash risk holds the equity that is most secure. That is the correct shape, and it falls out of the method rather than being imposed.

5. WHAT COMMITMENT ACTUALLY MEANS, AND WHO SETS THE NUMBER

- 5.1 The question of how often Dr Kpaduwa appears, and what happens if she does not, has been treated so far as a commercial one to be negotiated. It is not. For the surgical part of the business it is settled by a professional standard, and the standard is set by her own specialty body rather than by any party to this venture.
- 5.2 The American Society of Plastic Surgeons issued a Position Statement on Itinerant Surgery on 19 September 2024. Itinerant surgery is defined as a procedure performed by a surgeon at a distance from the surgeon's primary practice where the surgeon is not available for part or all of the patient's preoperative or postoperative care. The position is that **it is unethical for physicians to intentionally perform surgery under circumstances where they do not establish a relationship with the patient and or do not maintain responsibility for the continuity of the patient's care.** The surgeon must be involved in preoperative diagnosis and treatment planning, informed consent, performance of the procedure, and postoperative care including the treatment of surgical complications.
- 5.3 Delegation is permitted only in narrow circumstances, which ASPS lists as a remote area where subspecialty surgical care is unavailable, illness of the operating surgeon, live surgery at an accredited educational symposium, and disaster relief or humanitarian assistance. A commercial

clinic in Lagos is none of those. Where care is nonetheless transferred, the patient must be informed and the transfer should be to a board-certified plastic surgeon, or failing that to a qualified surgeon trained in the anatomical areas involved.

5.4 Two consequences follow and both matter to this venture.

(a) **The standard has teeth for Dr Kpaduwa personally.** ASPS states that members who perform itinerant surgery without maintaining responsibility for postoperative care, or who delegate that care to a provider not qualified to undertake it, may be refused membership, and that active members in violation face the Ethics Committee, the Judicial Council and possible expulsion. Designing this badly puts her standing at risk. Designing it properly protects it. That is why this section exists.

(b) **ASPS names our business model as the risk case.** It records that business models are deeply problematic where, as a means of lowering the cost of surgery to increase profit through higher case volume, they rely heavily on contracted surgeons to perform itinerant surgery. A venture built on a visiting surgeon has to answer that objection deliberately rather than hope nobody raises it.

5.5 The clinical window sets the length of a visit. Haematoma presents within 24 to 72 hours and can appear up to ten days after drains are removed. Infection presents typically between days three and fourteen. So the period over which the operating surgeon's involvement actually matters is about two weeks from the procedure, not two days.

5.6 That produces a hard design choice, and it is the choice that should be made now because it also decides who the Company hires.

Option	What it requires	Consequence
A. She covers alone	Operating early in each visit and remaining through the fourteen day window. In practice a visit of about two weeks, four times a year, which is roughly 56 days a year in Nigeria.	No resident surgeon needed. Heavy on her time, and fragile: one changed flight and the standard is breached.
B. A resident plastic surgeon covers	The Company recruits a board-certified or equivalently qualified plastic surgeon resident in Lagos. She operates, co-manages, and transfers care with the patient's informed consent. Visits can then be about 10 days, 3 times a year.	Meets the standard, is robust to travel, and is the only version that scales. Adds a senior salary to the operating model.

5.7 **We recommend option B, and that the appointment of the resident surgeon is a condition of the Company offering any surgical procedure at all.** The Company's present hiring plan is two surgical nurses and a care coordinator. That plan supports the non-surgical menu and does not support surgery. The commitment schedule and the recruitment plan are therefore the same decision, and it is better made now than discovered after the first case.

5.8 None of this constrains the non-surgical business, which is the whole of Stage 1. Injectables, skin, regenerative treatments and dermatology are delivered by Nigeria-registered clinicians to her protocols under a resident prescriber, and her presence is required for training, governance and complex cases rather than for safety cover.

5.9 On that basis the commitment schedule below is proposed. It is shaped by what is sustainable rather than by what sounds acceptable in a meeting, and it remains the Clinical Director's to accept or correct.

Party	Commitment	Measured by
Dr Chinwe Kpaduwa	3 visits a year, minimum 10 working days each under option B, with at least 2 operating lists per visit once registration permits. Four remote clinical governance sessions a month. Protocols reviewed and signed off quarterly. Named training milestones delivered for each clinician. Named as responsible surgeon for each of her cases, with any transfer of postoperative care made to the resident plastic surgeon on the patient's informed consent. An initial term renewed annually, with the operating calendar fixed a year ahead and two months of the year excluded by agreement.	Attendance recorded. Cases logged. Sign-offs dated. Transfers of care documented. Reported to the Board quarterly.
Consult for Africa	Incorporation and statutory registrations. Regulatory establishment including facility registration and the Clinical Director's licence process. The founding team recruited and in post, including the resident plastic surgeon before any surgical procedure is offered. Operating systems live. The first two quarters of operation delivered against an agreed plan.	Each is a dated deliverable. Vesting on written Board acceptance of each milestone.
Medbury Healthcare Group	Funding drawn to the approved budget and schedule. The base delivered and fitted out to programme. Group referral protocol in force and referrals actually routed to the Company.	Drawdowns evidenced. Practical completion certified. Referrals counted quarterly.

- 5.10 If a commitment is missed, the tranche it relates to does not vest in that period. It is not forfeited for good where the failure is temporary and explained, and the Board may carry it forward once. What it does not do is vest automatically.

6. IF A PARTY LEAVES

- 6.1 Unvested equity lapses. Vested equity is kept and is bought out on the valuation basis in the Shareholders' Agreement.
- 6.2 On a departure by Dr Kpaduwa the licence of her name terminates on an agreed wind-down and the Company continues under its own name, which is precisely why the Company was given a name of its own. **The Company keeps what it has been built out of:** the protocols as documented within it, the trained and credentialed team, its patient records and data, its systems and its content. Those were transferred, not lent.
- 6.3 On a departure by CFA the operating systems revert to CFA under the services agreement, and the Company keeps its records, its team and its documented processes. CFA's founding equity, being for work already delivered and accepted, is unaffected.
- 6.4 Bad leaver treatment, being forfeiture of unvested equity and a buy-out of vested equity at the lower of cost and value, applies only on defined and serious breach. The definition is deliberately narrow and belongs in the Shareholders' Agreement.

7. DECISION MAKING AND OWNERSHIP

- 7.1 The point has been made that greater investment and greater risk should carry corresponding authority. That is fair, and the current drafting does not fully reflect it. We propose the following, which gives Medbury the authority it is asking for without disturbing what made the structure acceptable to Dr Kpaduwa.

- (a) **Commercial decisions follow ownership.** On the operating matters reserved to the Board, being budget, pricing, appointments, contracts, premises and marketing spend, the chair appointed by Medbury has a casting vote. Medbury carries the commercial risk and should carry the commercial decision.
 - (b) **Clinical decisions do not.** The clinical standard, the protocols, credentialing, patient selection, clinical claims and the use of Dr Kpaduwa's name remain hers absolutely and are not voted on. No investor should hold a casting vote over a clinical judgement, and no clinician of standing would lend a name to a business where one did.
 - (c) **Fundamental matters stay unanimous.** The name, dilution, scope, sale, a new site and termination of the brand licence continue to require all three shareholders, which protects every party including Medbury.
- 7.2 That division is the ordinary one in clinician-led businesses. It costs Dr Kpaduwa nothing she was relying on, because her concern was never the marketing budget, and it gives Medbury the control its capital justifies.

8. THE FACILITY, AT MEDBURY'S OWN FIGURE

- 8.1 The review is right that the opportunity cost of the space was not separately valued, and it should have been. Taking Medbury's own figure of NGN 350,000 to NGN 375,000 per square metre and an indicative 200 square metres, the space is worth roughly NGN 70m to NGN 75m a year, or NGN 210m to NGN 225m over three years. The area is the input to confirm.
- 8.2 It should be treated exactly as Medbury proposed, as a rent cost in the profit and loss account. That keeps the Company's accounts honest and lets the campus recover its value. The consequence under the agreed principle is simple and cuts both ways: **rent that is paid is compensated and earns no equity; rent that is waived is a contribution and does.** Any waiver should be limited to the recovery of Medbury's own investment in the base, as proposed, and should be time-limited and stated in naira rather than open-ended.
- 8.3 A hybrid of a reduced rent plus a revenue share is workable and is common for anchor space. It should be modelled before it is agreed, because a revenue share behaves very differently from a fixed rent in the years when the Company is still filling its diary.

9. THE TEN QUESTIONS

	Question	Response
1	Valuation basis for every intangible	Agreed. A basis note will be supplied for each line, showing the market rate used, its source and the period.
2	Same methodology for Medbury's access and Dr K's demand	Agreed, and correct. Both will be restated on one basis, being acquisition cost avoided on the revenue each is shown to attract. Whatever that produces, it produces for both.
3	Breakdown of the corporate platform figure	Agreed. It will be itemised by function, with the market cost of buying each service in.
4	Medbury contributions excluded from the calculation	Conceded. The opportunity cost of the space was omitted and is dealt with at section 8. Governance architecture, quality systems and procurement leverage will be itemised and valued or expressly recorded as nil, with reasons.
5	Market comparables for the operating fee	Will be supplied. The point largely falls away because CFA now charges the market rate, so nothing turns on the comparison.
6	Why a fee discount should become equity	Conceded in full. It should not, and it no longer does. This was the best point in the review.
7	Separate founding contributions from ongoing service	Agreed. That separation is section 3 and is now the spine of the structure.
8	Vest future-service equity as delivered	Agreed. Sections 3 to 5.
9	Recalculate variable contributions on actual revenue	Agreed. The royalty and clinical components will be computed annually on actual revenue, and vesting will follow the actual figure, not the forecast.
10	Leaver and termination scenarios in the model	Agreed. Section 6, and carried into the Shareholders' Agreement.

10. THE TEST MEDBURY PROPOSED FOR ITSELF

- 10.1 The review asks what it would cost Medbury to hire equivalent clinical talent, build the brand and hire a management team if neither other party existed. It is the right test and we would encourage it, with one qualification.
- 10.2 Applied to management, the test is now answered by concession: CFA charges the market rate, so Medbury pays exactly what hiring a team would cost, and the equity is for founding work with dated deliverables which can be priced against what a firm would charge to do them.
- 10.3 Applied to the clinical side the test has a limit worth naming. A surgeon can be hired, and the surgeon fee pool of roughly NGN 168m over three years is what that costs. What cannot be hired is the name, the following it brings, or a standard already proven in this patient population, because those belong to a person rather than to a role. The correct conclusion from the test is therefore to pay the market rate for the service and to grant equity only for the part that could not have been bought, which is what the schedule at section 4 now does.

11. HOW THE REMAINING NUMBERS GET SETTLED

- 11.1 Several of the figures in this paper are ours rather than the Parties'. The honest way to replace them is to ask each principal the same questions privately and in writing, rather than to negotiate them in a group chat where nobody wants to be the first to state a limit.
- 11.2 A short structured questionnaire is being issued to Dr Kpaduwa and to Medbury. It asks each for the things only they can answer: for Dr Kpaduwa the visits and days she can genuinely sustain and for how long, the procedures she will and will not perform in Lagos, what she needs in a resident surgeon, and how she wishes to be paid against equity; for Medbury the funding ceiling and drawdown, the rent basis and any waiver, the referral volume it can actually commit, and the decisions over which it requires authority. Both are asked the same closing questions: what would make you walk away, and what does success look like at the November review.
- 11.3 This paper is a position, not a conclusion. It is being sent now because the work cannot wait for perfect information, and it will be revised on the answers.

12. WHAT WE NEED TO AGREE

- (a) That the classification at section 3 is the right one, and that equity is granted when the contribution is delivered.
- (b) The commitment schedule at section 5, in particular the number of visits, days and operating lists, which are ours to propose and Dr Kpaduwa's to accept.
- (c) The vesting periods, and whether earned equity vests quarterly or annually.
- (d) The division of decision rights at section 7.
- (e) The area of the clinic and the rent basis at section 8, and whether any waiver applies.
- (f) The Parties' own figures in place of every benchmark, at which point the schedule is rerun and the percentages settle themselves.